APPELLANT'S BRIEF

UNITED STATES COURT OF APPEALS FOR VETERANS CLAIMS

No. 20-7621

MONTE D. BUTLER,
Appellant,

v.

DENIS MCDONOUGH,
SECRETARY OF
VETERANS AFFAIRS,
Appellee.

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IN THE UNITED STATES COURT OF APPEALS FOR VETERANS CLAIMS

MONTE D. BUTLER, Appellant, v. DENIS MCDONOUGH, Secretary of Veterans Affairs, Appellee.	))))))))	Vet. App. No. 20-7621

BRIEF OF THE APPELLANT

Pursuant to U.S. Vet. App. R. 28 and 31, Monte D. Butler (Appellant or Veteran or Claimant) respectfully submits to the United States Court of Appeals for Veterans Claims (Court) the initial Brief of the Appellant stating that there are errors of law contained within the Department of Veterans Affairs (VA) decision of October 21, 2020 in which the Board of Veterans' Appeals (Board or BVA) denied the Appellant's claims of entitlement to an initial disability rating in excess of 30 percent prior to September 26, 2015 for service-connected sarcoidosis and in excess of 60 percent thereafter. In support of his position, Appellant relies on the information contained within the Record Before the Agency (RBA or R.) as filed with the Court and the following Brief of the Appellant.

This Honorable Court has jurisdiction to review the October 21, 2020 Board decision under the authority of 38 U.S.C. §§ 7252(a), 7261(a), and 7266.

STATEMENT OF THE ISSUE

I: WHETHER THE BOARD FAILED TO SUPPORT ITS OCTOBER 21, 2020 DECISION WITH ADEQUATE REASONS OR BASES.

STATEMENT OF THE CASE

I. Nature of the Case

The Veteran, Monte D. Butler, appeals an October 21, 2020 decision by the Board that denied his claims of entitlement to an initial disability rating in excess of 30 percent prior to September 26, 2015 for service-connected sarcoidosis and in excess of 60 percent thereafter. *See* Record (R.) at 1-16.

II. Course and Results of Proceedings Below

This appeal arises from a May 2016 rating decision. *See* R. at 1240-1241. The Veteran timely submitted his notice of disagreement (NOD) (R. at 1199-1200) and, after Decision Review Officer (DRO) review, perfected his appeal (R. at 859-860) leading to the further denial by the Board in a decision of October 21, 2020. *See* R. at 1-16.

III. Statement of the Facts

The Veteran, Monte D. Butler, served honorably on active duty in the United States United States Army from August 1996 to August 2000. *See* R. at 1425.

The present appeal stems from a May 2016 Rating Decision which granted the Veteran's claim of entitlement to service connection for sarcoidosis[1] and assigned a 30

[1] Sarcoidosis is defined as "a disease characterized by the growth of tiny collections of inflammatory cells (granulomas) in any part of your body- most commonly the lungs and lymph nodes. But it can also affect the eyes, skin, heart and other organs." www.mayoclinic.org/disease-conditions/sarcoidosis/symptoms-causes/syc-20350358 (last visited April 14, 2021).

percent disability rating effective March 23, 2010. *See* R. at 1240-1241. The Veteran timely filed a Notice of Disagreement (NOD) (R. at 1199-1200) with the rating assigned and then appealed to the Board of Veterans' Appeals (Board or BVA). *See* R. at 859-860.

Thereafter, in an April 2019 decision, the Board denied the Veteran's claim for an initial rating in excess of 30 percent for service-connected sarcoidosis. *See* R. at 168-180. The Veteran timely appealed to the U.S. Court of Appeals for Veterans Claims (Court or CAVC) and, pursuant to a Joint Motion for Remand (JMR) (R. at 40-44), the Court vacated the Board's April 2019 decision and remanded the matter. *See* R. at 45. In the JMR, the parties agreed that the Board had "… erred because it did not adequately consider favorable evidence of fevers and night sweats, weight loss and Appellant's use of corticosteroids when it denied a higher rating under 38 C.F.R. § 4.97, Diagnostic Code (DC) 6846." R. at 40.

Most recently, in an October 21, 2020 decision, the Board found that "[p]rior to September 26, 2015, the preponderance of the evidence shows that the Veteran's service-connected sarcoidosis more nearly approximates pulmonary involvement with persistent symptoms requiring chronic low dose (maintenance) or intermittent corticosteroids." R. at 5. Therefore, the Board concluded that "[t]he criteria for an initial rating in excess of 30 percent for service-connected sarcoidosis prior to September 26, 2015 are not met." R. at 6. However, the Board did find that

> [a]s of September 26, 2015, the evidence is at least equally balanced as to whether the Veteran's service-connected sarcoidosis more nearly approximates pulmonary involvement with persistent symptoms requiring

> systemic high dose (therapeutic) corticosteroids for control, with no evidence of cor pulmonale, cardiac involvement with congestive heart failure, or progressive pulmonary disease with fever, night sweats, and weight loss despite treatment.

R. at 5. As such, the Board concluded that "[t]he criteria for a disability rating of 60 percent, but not higher, for service-connected sarcoidosis from September 26, 2015 are met." R. at 6. A timely appeal to this Honorable Court followed.

ARGUMENT

Summary

The Appellant contends that the Board in this instance failed to consider all pertinent and favorable evidence of record thereby failing to support its October 21, 2020 decision with adequate reasons or bases.

I. THE BOARD FAILED TO SUPPORT ITS OCTOBER 21, 2020 DECISION WITH ADEQUATE REASONS OR BASES.

The Board is to include in its decision a statement of the reasons or bases for its findings and conclusions on all material issues of fact and law presented on the record, with such statement being adequate to enable an appellant to understand the precise basis for the Board's decision as well as to facilitate review by this Court. *See* 38 U.S.C. § 7104(d)(1); *see also Allday v. Brown*, 7 Vet. App. 517, 527 (1995); *Gilbert v. Derwinski*, 1 Vet. App. 49, 56-57 (1990). To fulfill this requirement, the Board must "… set forth the precise basis for its decision, [ ] analyze the credibility and probative value of all material evidence submitted by and on behalf of a claimant in support of the claim, and [ ] provide

a statement of its reasons or bases for rejecting any such evidence." *Gabrielson v. Brown*, 7 Vet. App. 36, 40 (1994). If the Board fails to provide an adequate statement of reasons or bases, remand is the appropriate remedy. *See Tucker v. West*, 11 Vet. App. 369, 374 (1998); *see also Washington v. Nicholson*, 19 Vet. App. 362, 371 (2005) (remand is the appropriate remedy when the Board fails to provide an adequate statement of reasons or bases for its determination).

VA is required to give due consideration to "all pertinent medical and lay evidence" in evaluating a claim for disability or death benefits. *See* 38 U.S.C. § 1154(a) (2006). The Board is not required to discuss all of the evidence of record, "but it cannot reject evidence favorable to the claimant without discussing that evidence." *Meyer v. Brown*, 9 Vet. App. 425, 433 (1996). Further, the Board must account for the evidence that it finds persuasive or unpersuasive, and its decisions must contain clear analysis, and succinct but complete explanations. *Gilbert*, 1 Vet. App. at 57.

The Veteran's service-connected sarcoidosis is evaluated under 38 C.F.R. § 4.97, Diagnostic Code (DC) 6846. Under diagnostic code 6846, a 30 percent rating is warranted for persistent symptoms requiring chronic low dose (maintenance) or intermittent corticosteroids. *See Id.* Sarcoidosis with pulmonary involvement requiring systemic high dose (therapeutic) corticosteroids for control is rated 60 percent disabling. *See Id.* Sarcoidosis with cor pulmonale, or; cardiac involvement with congestive heart failure, or; progressive pulmonary disease with fever, night sweats, and weight loss despite treatment, is rated 100 percent disabling. *See Id.*

In the instant case, the Board found that "[t]he medical evidence consistently reflects that the Veteran's sarcoidosis did not produce cor pulmonale[.]" R. at 14. However, the Board has overlooked or ignored medical evidence demonstrating that the Veteran's sarcoidosis does produce cor pulmonale. Specifically, the December 2010 VA doctor stated that the Veteran had a positive history for, inter alia, cor pulmonale. *See* R. at 1580 (1579-1580). This medical evidence is in direct contradiction to the Board's finding that there is no medical evidence reflecting that the Veteran's sarcoidosis produces cor pulmonale. *See* R. at 14. This is significant because sarcoidosis with cor pulmonale warrants a 100 percent disability rating. *See* 38 C.F.R. § 4.97, DC 6846.

Further, it is well documented in the Veteran's medical records that he also suffers from weight loss, fevers and night sweats. *See* R. at 860; *see also* R. at 1200; R. at 1580 (1579-1590); R. at 1156. Thus, the overlooked evidence coupled with these documented symptoms further supports entitlement to a 100 percent disability rating pursuant to 38 C.F.R. § 4.97, DC 6846. This is a narrowly based, albeit significant, point of contention and basis for remand. The medical evidence of record supports that the Veteran's sarcoidosis should be rated as 100 percent disabling.

Consequently, the Board failed to provide adequate reasons or bases to support its conclusion that "… an initial rating in excess of 30 percent prior to September 26, 2015 is not warranted and entitlement to a rating of 60 percent is warranted as of September 26, 2015." R. at 16. Accordingly, remand is warranted.

CONCLUSION

For the foregoing reasons, Appellant respectfully requests that the October 21, 2020 Board decision be set aside and the case remanded for further adjudication consistent with this Court's decision and applicable law.

Respectfully submitted,

April 14, 2021

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